

The Battered Child

Bruising is the result of blunt trauma, delivered either accidentally or intentionally. Active children, particularly toddlers, are prone to multiple bruises, and the identification of abusive injury is fraught with difficulty. The size, shape, color, and feel of a bruise varies based on anatomic site, the degree of force used, the firmness of the object delivering the force, and the underlying health of the injured individual. Great care and attention to detail must be exercised when evaluating these children, who are often brought to the office for an unrelated complaint. The history should include as much detail as possible regarding the onset, mechanism, and progression of injuries.

Conditions Mistaken for Abusive Bruising

- True petechiae and purpura
- Disorders of coagulation
- Ehlers-Danlos syndrome
- Infections
- Rocky Mountain spotted fever
- Meningococcal infections
- Group A streptococcal infections
- Palpable purpura of vasculitis
- Valsalva petechiae
- Lichen sclerosus
- Folk remedies
- Cao gio: vigorous rubbing with a hard object such as a coin
- Cupping: suction mark left by the cooling of a warm metal cup
- Nodular lesions mimicking deep bruises
- Neuroblastoma
- Vascular malformations
- Dermatomyositis-associated nodules

- Discolorations that look like bruises
- Phytophotodermatitis
- Maculae caeruleae from lice infestation
- Mongolian spots
- Dye from blue jeans
- Inflammatory conditions that mimic bruising
- Urticaria, angioedema, urticarial vasculitis
- Pernio
- Conditions that mimic whip marks
- Incontinentia pigmenti
- Striae
- Phytophotodermatitis

Conditions Mistaken for Nonaccidental Burns

- Phytophotodermatitis
- Folk remedies (Maquas, or moxibustion): burns delivered near diseased organs or therapeutic sites as in acupuncture
- Impetigo
- Epidermolysis bullosa
- Immunobullous diseases
- Sunburn, xeroderma pigmentosum
- Burns from objects heated by the sun
- Electric burn from an enuresis blanket
- Chemical burns from:
 - Undiluted acetic acid
 - Icy Hot balm
 - Calcium chloride
 - Diaper dermatitis
 - Pernio
 - Fixed drug eruption

Ideally, there should be an abuse team consisting of a dermatologist, pediatrician, social worker, medical photographer, and, when needed, pediatric subspecialists such as orthopedists, hematologists, psychologists, and gynecologists. It is most helpful if a relationship is established with the abuse team before an abuse incident occurs, and a clear protocol for managing suspected or alleged abuse is in place within the practitioner's office. Local emergency numbers for reporting child abuse can be obtained from the Child Welfare Information Gateway or Childhelp National Headquarters.

Child abuse affects children across all ages, races, and socioeconomic classes. However, children who are more vulnerable to abuse are typically between 5 and 15 years of age and may have emotional or behavioral problems, special medical needs, multiple siblings, live in single-parent households, or reside at or below the poverty level. Abuse is approximately twice as common among Pacific Islander, American Indian, Native Alaskan, and African American children compared to the general U.S. population. Perpetrators often have emotional or psychological difficulties, a history of being abused themselves, substance use disorders, a history of domestic violence or marital conflict, poor parenting skills, and low self-esteem. While these risk factors are informative, it is critical to remember that any child may be a victim of abuse, regardless of background or circumstance.